Blood 146 (2025) 834–835



The 67th ASH Annual Meeting Abstracts

ORAL

909. EDUCATION, COMMUNICATION, AND WORKFORCE

The impact of a clinical trial communication training workshop on hematology-oncology Fellows' knowledge, attitudes and behaviors: A mixed-methods evaluation

Carma Bylund[1], Susan Eggly[2], Daphne Friedman[3], Leah Szumita[4], Naomi Parker[1], Tithi Amin[1], Marie Michele Sainvil[3], Andrea Sitlinger[3], Stephanie Staras[1], Tuo Lin[1], Tara Hashemian[1], Elisa Weiss[4]

[1]University of Florida, Health Outcomes and Biomedical Informatics, Gainesville, United States
[2]Wayne State University Karmanos Cancer Institute, Detroit, United States
[3]Duke University School of Medicine, Department of Medicine, Durham, United States
[4]The Leukemia & Lymphoma Society, Rye Brook, United States

Abstract Introduction: Cancer clinical trials (CCTs) are essential to advancing the treatment of blood cancers and providing access to high-quality care. However, participation remains low, particularly among underrepresented groups such as patients who are older, who live in rural areas, are of a minoritized race and/or ethnicity, and who are adolescents or young adults. While many patients are interested in participating in CCTs, oncologists may inconsistently offer trials, use complex language, or make assumptions about patient interest or eligibility. Patient-centered communication training has the potential to improve oncologists' communication quality and comfort when speaking with patients about trials as a treatment option. We evaluated the impact of a communication skills workshop on Hematology-Oncology fellows' knowledge, attitudes/comfort, and behaviors related to discussing CCTs with patients.

Methods: We partnered with Blood Cancer United, formerly The Leukemia & Lymphoma Society, to develop a 3-hour workshop for Hematology-Oncology fellows. We implemented this workshop with seven fellowship programs. In the first hour of the workshop, multi-disciplinary faculty facilitators described barriers to CCT participation, introduced the COMM-CCT framework (*Check-in, Outline Options, Make a Shared Decision, Map Out Next Steps*), and introduced resources to identify available CCTs, such as the LLS Clinical Trial Support Center, a free clinical trial nurse navigation service. Next, fellows participated in a two-hour role play session to practice communication skills with blood cancer survivors acting as patients and received feedback from facilitators and peers. Before the implementation of the workshop, we invited fellows to participate in a study about the workshop that required completing pre-, post-, and 3- and 6-month follow-up surveys assessing CCT communication knowledge (8 true/false items), attitudes and comfort (10 items on a 5-point scale, 5=high), and behavior (i.e., number of patients with whom they discussed a trial within the past 3 months.) In the follow-up surveys, we asked what changes they had made to their practice since the training. Respondents were also invited to participate in a semi-structured interview following the training.

Results: 57 fellows completed the pre-training survey, 48 fellows completed the workshop, and 42 completed the post-training survey, with 36 and 34 completing 3- and 6-month follow-up surveys respectively. 14 fellows participated in the interviews. Participants had an average age of 33 years. Most participants in the study were identified as male (61.4%) and as White (56.4%). Knowledge scores were analyzed as percentage of items correct, and the mean percentage increased from pre- (76%) to post-workshop (79%), reaching significance at 3-month follow-up (83%, $p<.05$) and 6-month follow-up (81%, $p<.05$.). Mean scores on the attitudes and comfort measure increased significantly from pre- (3.9) to post-workshop (4.4, $p<.001$) and remained significant at 3-month (4.4, $p<.001$) and 6-month follow-up (4.5, $p<.001$). In the pre-training survey, 2% of participants reported that they had talked to 5 or more patients about CCTs in the preceding 3-month period. In the 3- and 6-month follow up surveys, 32% and 50% of fellows reported they had talked to 5 or more patients about CCTs in the preceding 3-month period. In the open-ended responses, participants reported making several changes to their practice, including discussing clinical trials earlier and more frequently, and with greater intentionality. These changes were facilitated by (1) increased comfort in trial discussions, (2) enhanced awareness of patient barriers and reservations, (3) improved preparation, and (4) stronger CCT communication skills. Interviews further illustrated shifts toward greater comfort with CCT communication, as participants reported learning that they did not need to be experts on every available trial in order to initiate trial discussions with patients. They also emphasized the value of having resources available to support patients who were interested in trials.

 © 2025 American Society of Hematology. Published by Elsevier Inc. All rights reserved.

Conclusion: These findings support the COMM-CCT training workshop as an effective and potentially scalable educational intervention within fellowship trainings across the U.S. that provides a way to improve cancer clinical trial communication. Future work should examine the impact of the work on fellows' practice long-term and on accrual.

Disclosure Sitlinger: *Beigene:* Consultancy (Includes expert testimony) *DAVA Oncology:* Consultancy (Includes expert testimony); Ended employment in the past 24 months *Genmab:* Research Funding *Loxo/Lilly:* Research Funding

https://doi.org/10.1182/blood-2025-834